Physician Order Form

Facility Iceberg health

Patient Name Fatai Okin DOB 04/10/1980

Home # _____ Cell # (404) 457-4115 Work # _____

Attorney Name: Morgan & Morgan - Atlanta Office # (404) 965-8811 Fax # _____

Referring Physician Dr Marcia Smith D.C Office # 678-505-0000 Fax # 470-823-9759

Type of Injury MVA DOL 05/27/2024

By signing below, I certify that all requested tests are medically necessary and authorize the imaging center to contact patient for scheduling and to obtain all information necessary for testing to be completed.

Physician Signature _____

.....

Stat Telephone Report ☐ Yes ☐ No Fax Report Fax # _____

Report / Films ☑ Report Only ☐ Films with Patient ☐ Films Delivered ☐ CD-ROM

Diagnosis (Please include ICD-10 code) _____

Sprain in ligament in cervical and Thoracic spine

Special Instructions

_____

.....

MRI ☐ **W/ Contrast** ☑ **W/O Contrast** ☐ **3D Rendering if positive** **Extremities**

☐ Brain	☑ Cervical Spine	☐ Shoulder	Left/Right
☐ Orbits	☑ Thoracic Spine	☐ Elbow	Left/Right
☐ Pituitary/Sella	☐ Lumbar Spine	☐ Forearm	Left/Right
☐ IAC's	☐ Brachial Plexus	☐ Wrist	Left/Right
☐ TMJ's	☐ Lumbar Plexus	☐ Hand/Fingers	Left/Right
☐ Stroke Protocol (MRI Brain,	☐ Soft Tissue Neck	☐ Hips	Left/Right
☐ MRA Head, MRA Neck)	☐ Chest	☐ Femur	Left/Right
☐ MRCP	☐ Abdomen	☐ Knee	Left/Right
☐ Other _____	☐ Pelvis	☐ Tib/Fib	Left/Right
		☐ Ankle/Hindft	Left/Right
		☐ Midfoot/ Toes	Left/Right

PATIENT:	OKIN, FATAI
DOB:	4/10/1980
FILE #:	RAM260370
PHYSICIAN:	SMITH, MARCIA
EXAMS:	MRI THORACIC SPINE W/O CONTRAST
DATE:	6/9/2024 1:02:05 PM

MRI OF THE THORACIC SPINE:

HISTORY: Mid back pain following a motor vehicle collision dated 05/27/2024.

TECHNIQUE: Multisequence T1 and T2 weighted images were obtained.

FINDINGS: Evaluation of the thoracic spine demonstrates normal kyphotic curvature. No evidence for abnormal mass or fluid collection is seen. There is no evidence for fracture. There is no evidence for prevertebral or paravertebral mass or fluid collection. Examination of the thoracic cord demonstrates that there is normal signal. The adjacent soft tissues demonstrate no significant abnormalities.

Segmental analysis of the thoracic spine is as follows:

At T1-2, there is no evidence for disc herniation, canal stenosis, or foraminal stenosis.

At T2-3, there is no evidence for disc herniation, canal stenosis, or foraminal stenosis.

At T3-4, there is no evidence for disc herniation, canal stenosis, or foraminal stenosis.

At T4-5, there is no evidence for disc herniation, canal stenosis, or foraminal stenosis.

At T5-6, there is no evidence for disc herniation, canal stenosis, or foraminal stenosis.

At T6-7, there is no evidence for disc herniation, canal stenosis, or foraminal stenosis.

At T7-8, there is no evidence for disc herniation, canal stenosis, or foraminal stenosis.

At T8-9, there is no evidence for disc herniation, canal stenosis, or foraminal stenosis.

At T9-10, there is no evidence for disc herniation, canal stenosis, or foraminal stenosis.

At T10-11, there is no evidence for disc herniation, canal stenosis, or foraminal stenosis.

At T11-12, there is no evidence for disc herniation, canal stenosis, or foraminal stenosis.

Incidentally, there is a cyst in the right kidney that measures approximately 1.0 cm that is not fully evaluated on this MRI of the thoracic spine. If further evaluation is clinically desired, recommend a dedicated ultrasound.

IMPRESSION:

1. Unremarkable MRI of the thoracic spine.
2. Incidentally, there is a cyst in the right kidney that measures approximately 1.0 cm that is not fully evaluated on this MRI of the thoracic spine. If further evaluation is clinically desired, recommend a dedicated ultrasound.

The definitions in this report, including definitions of disc bulge, herniation, protrusion, and extrusion, are from the following peer reviewed journal: Disc Nomenclature V2.0, Recommendations of the Combined Task Forces of the North American Spine Society, the American Society of Spine Radiology and the American Society of Neuroradiology, The Spine Journal 14 (2014) 2525-2545. References to causation and permanency follow guidelines established by the American Medical Association. Note that a normal MRI does not exclude certain pathologies, including pathologies involving the nerves and facet joints. A normal MRI should not supersede abnormalities detected with physical exam. Disc herniations are contained herniated discs unless specifically identified as uncontained.

Thank you for your kind referral.

SEAN M. MAHAN, M.D.
ELITERAD RADIOLOGY GROUP
CERTIFIED, AMERICAN BOARD OF RADIOLOGY

SHAHIN J. KORANGY, M.D.
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SMM/TB

PATIENT:	OKIN, FATAI
DOB:	4/10/1980
FILE #:	RAM260369
PHYSICIAN:	SMITH, MARCIA
EXAMS:	MRI CERVICAL SPINE W/O CONTRAST
DATE:	6/9/2024 12:47:09 PM

MRI OF THE CERVICAL SPINE:

HISTORY: Neck pain following a motor vehicle collision dated 05/27/2024.

TECHNIQUE: Multisequence T1 and T2 weighted images were obtained.

FINDINGS: The posterior fossa structures are normal. The cervical cord structures are normal. The lordotic curvature is preserved. No prevertebral or paravertebral masses or fluid collections are identified.

There is edema in between the clivus and the odontoid process and this tracks along the alar ligaments. This finding is associated with whiplash associated disorder and is consistent with injury to the alar ligaments. There is no evidence of ligament rupture.

Segmental analysis of the cervical spine is as follows:

At C2-3, there is bulging of the disc. This results in an anterior impression on the thecal sac. There is no central canal stenosis or foraminal stenosis.

At C3-4, there is bulging of the disc. This results in an anterior impression on the thecal sac. There is no central canal stenosis or foraminal stenosis.

At C4-5, there is bulging of the disc. This results in an anterior impression on the thecal sac. There is no central canal stenosis or foraminal stenosis.

At C5-6, there is anterior disc bulging. There is no evidence for disc herniation, spinal canal, or neural foraminal stenosis.

At C6-7, there is no evidence for disc herniation, canal stenosis or neural foraminal stenosis.

At C7-T1, there is no evidence for disc herniation, canal stenosis or neural foraminal stenosis.

IMPRESSION:

1. There is edema in between the clivus and the odontoid process and this tracks along the alar ligaments. This finding is associated with whiplash associated disorder and is consistent with injury to the alar ligaments. There is no evidence of ligament rupture. **See figure 1, image 7, series 5.** This is a STIR image where fluid is bright. The circled arrow is pointing to the edema anterior to the tectorial membrane at the craniocervical junction.
2. At C2-3, there is bulging of the disc. This results in an anterior impression on the thecal sac.
3. At C3-4, there is bulging of the disc. This results in an anterior impression on the thecal sac.
4. At C4-5, there is bulging of the disc. This results in an anterior impression on the thecal sac.
5. At C5-6, there is anterior disc bulging.
6. Given the patient's history and findings, it is medically probable that the edema anterior to the tectorial membrane at the craniocervical junction is related to the injury sustained from the 05/27/2024 motor vehicle collision. Clinical correlation is recommended to confirm acute ligament injury at the craniocervical junction.

FIGURE 1

The definitions in this report, including definitions of disc bulge, herniation, protrusion, and extrusion, are from the following peer reviewed journal: Disc Nomenclature V2.0, Recommendations of the Combined Task Forces of the North American Spine Society, the American Society of Spine Radiology and the American Society of Neuroradiology, The Spine Journal 14 (2014) 2525-2545. References to causation and permanency follow guidelines established by the American Medical Association. Note that a normal MRI does not exclude certain pathologies, including pathologies involving the nerves and facet joints. A normal MRI should not supersede abnormalities detected with physical exam. Disc herniations are contained herniated discs unless specifically identified as uncontained.

Thank you for your kind referral.

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SMM/TB

PATIENT
Fatai Okin

DOB	04/10/1980
AGE	44 yrs
SEX	Unspecified
PRN	OF975077

FACILITY
Star Ortho and Spine of Georgia
T (404) 348-2750
F (404) 900-3864
1720 Phoenix Blvd.
Suite 800
Atlanta, GA 30349

ENCOUNTER
Office Visit

NOTE TYPE	SOAP Note
SEEN BY	Jordan Kuppinger MD
DATE	06/27/2024
AGE AT DOS	44 yrs

Electronically signed by Jordan Kuppinger MD at 07/10/2024 01:17 pm

Chief complaint

No chief complaint recorded

Subjective

Procedures:

1. Initial Consultation

Chief Complaint:

1. Neck pain
2. Back pain
3. Headaches

History of Present Illness:

Mr. Fatai Okin is a 44 yrs male with a history of a motor vehicle accident. The patient was T-boned which caused his car to spin on 5/27/2024. The patient was the driver and was restrained by a seatbelt. The vehicle was totaled.

Mr. Fatai Okin presents with 60% pain in the cervical spine and 40% numbness and tingling to his fingers. The patient's cervical pain is intermittent and is aggravated by movement. The quality of the patient's cervical pain is sharp, stabbing, and deep. The patient has 1 headache per week with an intensity of 5/10.

Mr. Fatai Okin presents today with 95% pain in the lumbar spine and 5% numbness and tingling to the legs. The patient does have pain in the right buttock. The patient's lumbar pain is intermittent and is aggravated by waking up in the morning and certain stretches at the chiropractor. The quality of the patient's lumbar pain is pulsating.

The pain has affected the patient's activities of daily living and the ability to sleep is sometimes interrupted but not consistent. The patient's cervical pain level today is 0/10 and at the worst in the last week 5/10. The patient's lumbar pain level today is 2/10 and at the worst in the last week 5/10. The patient does not take any pain medication. The patient has received treatment by a chiropractor 1 month and it helped, but has not relieved symptoms completely. The patient has not had interventional pain management procedures.

Objective

Blood Thinners: None

Allergies: No Known allergies to latex, tape, IV contrast, iodine

Social History:
Delivery Driver, Used Car Dealer
Married
Social alcohol, denies tobacco use.

Review of System: Patient denies any fever, chills, headaches, chest pain, shortness of breath, bowel or bladder incontinence and/or weakness in the upper or lower extremities.

Assessment

MRI IMPRESSION OF THE CERVICAL SPINE: DATED 06/09/24

1. There is edema in between the clivus and the odontoid process and this tracks along the alar ligaments. This finding is associated with whiplash associated disorder and is consistent with injury to the alar ligaments. There is no evidence of

ligament rupture. See figure 1, image 7, series 5. This is a STIR image where fluid is bright. The circled arrow is pointing to the edema anterior to the tectorial membrane at the craniocervical junction.

2. At C2-3, there is bulging of the disc. This results in an anterior impression on the thecal sac.
3. At C3-4, there is bulging of the disc. This results in an anterior impression on the thecal sac.
4. At C4-5, there is bulging of the disc. This results in an anterior impression on the thecal sac.
5. At C5-6, there is anterior disc bulging.
6. Given the patient's history and findings, it is medically probable that the edema anterior to the tectorial membrane at the craniocervical junction is related to the injury sustained from the 05/27/2024 motor vehicle collision. Clinical correlation is recommended to confirm acute ligament injury at the craniocervical junction.

PHYSICAL EXAMINATION:

General: Well-developed and well-nourished, in moderate discomfort.
HEENT: Head atraumatic, normocephalic. Eyes and ears without discharge. Nose without obstruction. Sclerae clear and anicteric. Hearing intact to spoken voice. Oral mucosa moist.
Heart: Regular rate and rhythm.
Lungs: Breathing even and unlabored. No audible abnormal breath sounds.
Abdomen: Nondistended.
Integument: No rashes or lesions identified.
Lymphatic: No lymphedema.
Extremities: No clubbing or cyanosis.
Vascular: Peripheral pulses are palpable.
Neuro: Alert and oriented.
Musculoskeletal: There is no tenderness in the calves.

PHYSICAL EXAM CERVICAL SPINE:

Inspection: Local inspection shows no step-off or bruising. Cervical alignment is normal.

Palpation: No tenderness over the midline

Right: There is **Mild** tenderness and spasm to palpation of right paraspinous muscle, upper trapezius muscle, middle trapezius/rhomboids muscle.

Left: There is no tenderness and spasm to palpation of left paraspinous muscle, suboccipital muscle, upper trapezius muscle, middle trapezius/rhomboids muscle, levator scapulae muscle and supraspinatus muscle.

Range of Motion: Is normal in flexion, left axial rotation, and left lateral flexion. Is **Decreased** in extension, right axial rotation, right lateral flexion.

Strength:

Right:	**Left:**
Deltoid (Axillary C5,C6) 5/5	Deltoid (Axillary C5,C6) 5/5
Biceps (Musculocutaneous C5-7) 5/5	Biceps (Musculocutaneous C5-7) 5/5
Triceps (Radial C6-8) 5/5	Triceps (Radial C6-8) 5/5
Wrist Extension 5/5	Wrist Extension 5/5
Wrist Flexors 5/5	Wrist Flexors 5/5

Sensation: Sensation is normal to touch and pinwheel except when noted.

Right:	**Left:**
Posterior aspect of shoulder (C4) Normal	Posterior aspect of shoulder (C4) Normal
Lateral aspect of upper arm (C5) Normal	Lateral aspect of upper arm (C5) Normal
Tip of thumb (C6) Normal	Tip of thumb (C6) Normal
Tip of middle finger (C7) Normal	Tip of middle finger (C7) Normal
Tip of pinky (C8) Normal	Tip of pinky (C8) Normal

Reflexes:

Right:	**Left:**
Biceps (C5,C6) 2+	Biceps (C5,C6) 2+

Brachioradialis (C6) 2+
Triceps (C7) 2+

Brachioradialis (C6) 2+
Triceps (C7) 2+

Facet Tenderness:
Right: Positive C3-4, C4-5
Left: Negative

Facet Loading:
Right: Positive C3-4, C4-5
Left: Negative

Special Tests:

Right:
Hoffman: Negative
Spurling's Sign: Negative
Lhermit's: Negative
Babinski: Normal
Clonus: Absent

Left:
Hoffman: Negative
Spurling's Sign: Negative
Lhermit's: Negative
Babinski: Normal
Clonus: Absent

Gait: Gait and station are normal. The patient ambulates unassisted.

Neurological: Denies any urinary or fecal incontinence

Grip Strength:
Right: 5/5
Left: 5/5

PHYSICAL EXAM LUMBAR:

Inspection: Local inspection shows no step-off or bruising. Lumbar alignment is normal

Palpation: No tenderness over the midline

Right:
Mild evidence of tenderness and spasm of the right lumbar paraspinous muscles.
No tenderness over the right piriformis muscles.
No tenderness noted to palpation over the right sacroiliac joints noted.
No tenderness noted over the right greater trochanter.

Left:
Mild evidence of tenderness and spasm of the left paraspinous muscles.
No tenderness over the left piriformis muscles.
No tenderness noted to palpation the left sacroiliac joints noted.
No tenderness noted over the left greater trochanter.

Range of Motion: Is normal in flexion. Is **Decreased** in extension, right axial rotation, right lateral flexion, left axial rotation and left lateral flexion.

Strength:

Right:
Iliopsoas (L2,L3) 5/5
Ankle dorsiflexion (L4) 5/5
Extensor hallucis longus (L5) 5/5
Plantar flexion (S1) 5/5

Left:
Iliopsoas (L2,L3) 5/5
Ankle dorsiflexion (L4) 5/5
Extensor hallucis longus (L5) 5/5
Plantar flexion (S1) 5/5

Sensation: Sensation is normal to touch and pinwheel except when noted.

Right:
Groin (L1) Normal

Left:
Groin (L1) Normal

Upper leg (L2) Normal	Upper leg (L2) Normal
Lower-medial upper leg (L3) Normal	Lower-medial upper leg (L3) Normal
Medial portion foot (L4) Normal	Medial portion foot (L4) Normal
Lateral lower leg (L5) Normal	Lateral lower leg (L5) Normal
Sole of foot (S1) Normal	Sole of foot (S1) Normal

Reflexes:

Right:	**Left:**
Patellar reflex (L4) 2+	Patellar reflex (L4) 2+
Achilles reflex (S1) 2+	Achilles reflex (S1) 2+
Clonus absent	Clonus absent
Babinski downward hallux	Babinski downward hallux

Facet Tenderness:
Right: Positive L4-5, L5-S1
Left: Positive L4-5, L5-S1

Facet Loading:
Right: Positive L4-5, L5-S1
Left: Positive L4-5, L5-S1

SPECIAL/PROVOCATION TESTS

Nerve Root irritation:

Right:	**Left:**
Kemp's Test: Negative	Kemp's Test: **Positive**
Slump test: Negative	Slump test: **Positive**
Straight leg raise: Negative	Straight leg raise: **Positive**
Femoral Stretch Test: Negative	Femoral Stretch Test: **Positive**

Sacral Iliac Joint:

Right:	**Left:**
Patrick's: Negative	Patrick's: Negative
Sacral compression: Negative	Sacral compression: Negative
Fortin: Negative	Fortin: Negative

Piriformis:

Right:	**Left:**
FAIR test: Negative	FAIR test: Negative

Reflexes:

Right:	**Left:**
Ankle Clonus: Negative	Ankle Clonus: Negative
Hoffman: Negative	Hoffman: Negative
Babinski: Negative	Babinski: Negative

Gait: Gait and station are normal. The patient ambulates unassisted.

Neurological: Denies any urinary or fecal incontinence

Diagnoses attached to this encounter:

Cervicalgia [ICD-10: M54.2], [ICD-9: 723.1], [SNOMED: 81680005]

Spondylosis without myelopathy or radiculopathy, cervical region [ICD-10: M47.812], [ICD-9: 721.0], [SNOMED: 267970006]

Low back pain, unspecified [ICD-10: M54.50], [SNOMED: 279039007]

Spondylosis without myelopathy or radiculopathy, lumbar region [ICD-10: M47.816], [ICD-9: 721.3], [SNOMED: 68859000]

Spondylosis without myelopathy or radiculopathy, lumbosacral region [ICD-10: M47.817], [ICD-9: 721.3], [SNOMED: 48210000]

Radiculopathy, lumbar region [ICD-10: M54.16], [ICD-9: 724.4], [SNOMED: 128196005]

Radiculopathy, lumbosacral region [ICD-10: M54.17], [ICD-9: 724.4], [SNOMED: 2415007]

Plan

Plan and Discussion:

1. The cervical MRI report was reviewed and discussed with the patient.
2. Continue conservative treatment including physical therapy, chiropractic care and/or OTC medications.
3. If these symptoms are left untreated, it may result in musculoskeletal impairment including permanent and significant pain symptomatology, nerve dysfunction, myofascial and tissue scarring; which can cause significant and permanent limitations in their range of motion, strength, coordination, balance and proprioception.
4. I believe within a reasonable degree of medical certainty that the above injuries were caused by or aggravated by the accident on 5/27/2024. I hereby attest that I am a licensed physician and that the above facts are true and correct.
5. Ibuprofen 800 mg #28 P.O. Q 6 hours prn pain. NR.
6. Flexeril 10 mg #30 P.O. Q 8 hours PRN muscle spasm. NR.
7. Follow up in one month.

practicefusion

Iceberg Health Morrow
1544 Southlake Pkwy
Unit 9F
Morrow, GA 30260-0287
(678) 505-0000

Initial Exam

Patient Name: Fatai Okin
Date of Birth: 4/10/1980

Provider: Marcia Smith, DC
Date of Service: 5/30/2024

SUBJECTIVE

HISTORY OF PRESENT ILLNESS

He has never experienced this condition before.

PRIMARY COMPLAINT

Low back, Mid back

- Burning, Aching, Sharp pain: 8/10
  Radiates to (anterior) left upper leg (thigh), right upper leg (thigh) and right lower leg.
  Described as constant (up to 100% of day).

SECOND COMPLAINT

Neck, Upper back

- Throbbing, Sharp pain: 8/10
  Radiates to (posterior) left shoulder and right shoulder.

THIRD COMPLAINT

Left chest, Right chest

- Aching pain: 6/10
  Described as constant (up to 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL).

FOURTH COMPLAINT

Head

- Aching, Throbbing pain: 4/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate intensity (requires significant or complete modification to ADL).

The patient reports a previous injury: 1.

PAST, FAMILY, SOCIAL HISTORY

Surgeries/Hospitalizations: None reported
Allergies: None reported
Medications: None reported
Prior conditions: None

REVIEW OF SYSTEMS

CARDIOVASCULAR: Chest pain and Dizziness

MUSCULOSKELETAL: Muscle pain, Neck pain, Back pain, Head injury, Back injury and Neck injury
NEUROLOGICAL: Headaches
PSYCHIATRIC: Difficult sleeping

The following are negative: ALLERGIC-IMMUNOLOGIC, CONSTITUTIONAL, EAR/NOSE/THROAT, ENDOCRINE, EYES, GASTROINTESTINAL, GENITOURINARY, HEMATOLOGY/LYMPH, RESPIRATORY and SKIN.

Collision occurred on 5/27/2024 at approximately 4:05 AM.
An SUV driver lost control of his SUV ran into my car and ran my car off the road.
Symptoms reported: Severe body pain (my chest, back, right leg, both shoulders, my hips)

Mr. Okin was the driver of the vehicle, a 2015 Toyota Corolla sport, at the time of the collision. There was rear, front, driver side, fender damage to the vehicle. The collision happened while he was moving at I - 20 west in Atlanta , GA. The weather was cloudy. Street surface was dry. The impact on the vehicle was at the front. Upon impact, the brakes were locked, and the car moved over 10 feet. Mr. Okin was seated in the vehicle as a driver and was wearing a seat belt. The vehicle is equipped with air bags. Mr. Okin saw the impact coming and he braced for the impact. Upon impact, his head was looking ahead and he was thrown Thrown around . His head hit dashboard . There was head trauma. Mr. Okin remembers the collision happening.
C/o pain that is radiating into his legs bilaterally that stops at his knees and pain on the right leg that goes into his right lower leg. Pt c/o headaches, neck pain, thoracic pain, chest pain and lumbar pain.

OBJECTIVE

VITALS
Height: 64 in Weight: 153 lbs BMI: 26.3
Pulse rate: 61 BPM (relaxed)
Blood Pressure (seated): right -N/A left-111/73 mmHg

PALPATORY FINDINGS
Spinal asymmetry/fixation: OCC; C1; C2; C5; C6; C7; T1; T4; T5; T6; T7; T10; T11; T12; L4; L5; Sacrum; SI.

Tenderness

Left Side
- Cervical Spine (moderate-severe)
- Upper Trapezius (moderate-severe)
- Thoracic Spine (moderate-severe)
- Lower Back (moderate-severe)

Right Side
- Cervical Spine (moderate-severe)
- Upper Trapezius (moderate-severe)
- Thoracic Spine (moderate-severe)
- Lower Back (moderate-severe)

Spasm/Hypertonicity

Left Side
- Cervical Spine (moderate-severe)
- Upper Trapezius (moderate-severe)

Right Side
- Cervical Spine (moderate-severe)
- Upper Trapezius (moderate-severe)

- Thoracic Spine (moderate-severe)
- Lower Back (moderate-severe)

- Thoracic Spine (moderate-severe)
- Lower Back (moderate-severe)

GAIT

Ambulation: Normal.
Gait: Normal.

ORTHOPEDIC AND NEUROLOGICAL TESTS

Test	Left	Right	Comment
Foraminal compression test[1]	Positive	Positive	
Shoulder depression test[2]	Positive	Positive	
Maximal foraminal compression test[3]	Positive	Positive	
Spinal percussion test-thoracic[4]	Positive	Positive	
Kemp's test[5]	Positive	Positive	
Milgram's test[6]	Positive	Positive	
Straight leg raise[7]	Positive	Positive	
Patrick test[8]	Positive	Positive	

[1]Radicular pain could indicate pressure on the nerve root. Localized pain could indicate sprain or strain or foraminal encroachment.

[2]Pain in the arm could indicate adhesions of the nerve roots. Local pain could indicate sprain or strain or adhesions of the shoulder joint capsule.

[3]Increased peripheral pain could indicate radiculopathy or IVF encroachment. Local neck pain could indicate joint or muscle damage.

[4]Localized pain suggests possible vertebral fracture. Radicular pain indicates possible disc lesion.

[5]Perhipheral pain in to the lower extremities indicates nerve root compression. Local pain may indicate sprain or strain.

[6]Lower back pain may indicate a herniated disc.

[7]Perhipheral pain is suggestive of nerve root irritation, such as that due to a herniated disc.

[8]Pain in the hip during abduction and external rotation may indicate a coxa pathologic condition.

Apley scratch test within normal limits.

REFLEXES

Location	Left	Right	Comment
Brachioradialis	1/4	1/4	
Patellar	1/4	1/4	

0-Absent; 1-Diminished; 2-Normal; 3-Exaggerated; 4-Hyper-Reflexive.

Biceps within normal limits bilaterally.

RANGE OF MOTION

Cervical ROM - estimated

Test	Measurement	Limited By	Comment
Flexion	25° (50% of normal)	Pain, Spasm, Guarding	

Extension	30° (50% of normal)	Pain, Spasm, Guarding	
Left lateral flexion	30° (66% of normal)	Pain, Spasm, Guarding	
Right lateral flexion	25° (55% of normal)	Pain, Spasm, Guarding	
Left rotation	55° (68% of normal)	Pain, Spasm, Guarding	
Right rotation	55° (68% of normal)	Pain, Spasm, Guarding	

Lumbar ROM - estimated

Test	Measurement	Limited By	Comment
Flexion	15° (25% of normal)	Pain, Spasm, Guarding	
Extension	5° (20% of normal)	Pain, Spasm, Guarding	
Left lateral flexion	10° (40% of normal)	Pain, Spasm, Guarding	
Right ateral flexion	10° (40% of normal)	Pain, Spasm, Guarding	
Left rotation	20° (66% of normal)	Pain, Spasm, Guarding	

Right rotation within normal limits.

MUSCLE STRENGTH

Upper Muscle Strength

Shoulder abductors, Shoulder adductors, Shoulder flexors, Shoulder extensors, Elbow flexors and Elbow extensors are within normal limits bilaterally.

Lower Muscle Strength

Location	Left	Right	Comment
Hip extensors	4/5	WNL	
Knee extensors	4/5	4/5	
Knee flexors	4/5	4/5	

0-No-muscle-activity; 1-Flicker-of-movement; 2-Movement-with-gravity-eliminated; 3-Movement-against-gravity-only; 4-Movement-against-gravity-with-some-resistance; 5-Movement-against-gravity-with-full-resistance.

Hip flexors within normal limits bilaterally.

DERMATOMES

Anterior

Left-Cervical, Right-Cervical, Left-Lumbar and Right-Lumbar are within normal limits.

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Radiculopathy, cervical region (M54.12)

2. Radiculopathy, lumbosacral region (M54.17)
3. Segmental and somatic dysfunction of lumbar region (M99.03)
4. Segmental and somatic dysfunction of cervical region (M99.01)
5. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
6. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
7. Disorder of ligament, vertebrae (M24.28)
8. Strain of muscle, fascia and tendon at neck level, init (S16.1XXA)
9. Sleep deprivation (Z72.820)
10. Strain of muscle, fascia and tendon of lower back, init (S39.012A)
11. Cervicalgia (M54.2)
12. Low back pain (M54.5)
13. Other muscle spasm (M62.838)
14. Segmental and somatic dysfunction of thoracic region (M99.02)
15. Person injured in unsp motor-vehicle accident, traffic, init (V89.2XXA)
16. Acute post-traumatic headache, intractable (G44.311)

LONG-TERM GOALS

- Increase cervical range of motion.
- Enable patient to sit without pain.
- Enable patient to stand without pain.
- Enable patient to sleep without being disturbed by pain.
- Enable patient get get up/sit down without pain.
- Increase pain-free ROM.
- Promote soft tissue healing.
- Relieve pain and prevent recurrence.
- Decrease anxiety.
- Decrease pain to pre-injury status.
- Restore tolerance to normal activities of daily living.
- Be able to perform normal ADL's.
- Be able to work pain- free.
- Increase lumbar range of motion.
- Increase thoracic range of motion.

PLAN

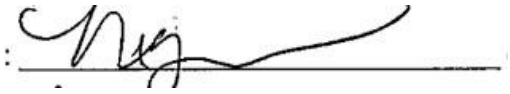
Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of diversified technique was provided specifically to OCC; C1; C2; C5; C6; C7; T1; T4; T5; T6; T7; T10; T11; T12; L4; L5; Sacrum; SI, to reduce pain reported at initial encounter by 25%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to Neck, to reduce pain reported at initial encounter by 25%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Back, to reduce pain reported at initial encounter by 25%.
- Ice/Heat (97010) consisting of ice pack, hot pack was provided specifically to Neck, Back,

to reduce pain reported at initial encounter by 25%.

- New Patient evaluation and management service, 99203, was provided today.

Patient received care without incident.



(Electronic Signature)
Provider Name: Marcia Smith, DC
Date: 05/30/2024 04:40 PM

Iceberg Health Morrow
1544 Southlake Pkwy
Unit 9F
Morrow, GA 30260-0287
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Fatai Okin
Date of Birth: 4/10/1980

Provider: Marcia Smith, DC
Date of Service: 6/4/2024

SUBJECTIVE

PRIMARY COMPLAINT
Low back, Mid back

- Burning, Aching, Sharp pain: **From 8 to 9/10 (10% increase)**
  Radiates to (anterior) **left upper leg (thigh)**, **right upper leg (thigh)** and **right lower leg**.
  Described as constant (up to 100% of day).

SECOND COMPLAINT
Neck, Upper back

- Throbbing, Sharp pain: 8/10
  Radiates to (posterior) left shoulder and right shoulder.

THIRD COMPLAINT
Left chest, Right chest

- Aching pain: 6/10
  Described as constant (up to 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL).

FOURTH COMPLAINT
Head

- Aching, Throbbing pain: 4/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate intensity (requires significant or complete modification to ADL).

Reports losing his dad last week and not being able to take the trip back to Nigeria due to such painful low back pain. **Reports it is a 13 hour flight and** that **he can't imagine traveling** that **long with the intense pain**. **Standing from sitting or laying down causes pain**. **Reports increased** pain **when getting out of bed**.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: OCC; C1; C2; C5; C6; C7; T1; T4; T5; T6; T7; T10; T11; T12; L4; L5; Sacrum; SI**.**

Tenderness

Left Side	Right Side

- Cervical Spine (Moderate-Severe)
- Upper Trapezius (Moderate-Severe)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)

- Cervical Spine (Moderate-Severe)
- Upper Trapezius (Moderate-Severe)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Moderate-Severe)
- Upper Trapezius (Moderate-Severe)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)

Right Side

- Cervical Spine (Moderate-Severe)
- Upper Trapezius (Moderate-Severe)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Radiculopathy, cervical region (M54.12)
2. Radiculopathy, lumbosacral region (M54.17)
3. Segmental and somatic dysfunction of lumbar region (M99.03)
4. Segmental and somatic dysfunction of cervical region (M99.01)
5. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
6. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
7. Disorder of ligament, vertebrae (M24.28)
8. Strain of muscle, fascia and tendon at neck level, init (S16.1XXA)
9. Sleep deprivation (Z72.820)
10. Strain of muscle, fascia and tendon of lower back, init (S39.012A)
11. Cervicalgia (M54.2)
12. **Other muscle spasm (M62.838)**
13. **Segmental and somatic dysfunction of thoracic region (M99.02)**
14. **Person injured in unsp motor-vehicle accident, traffic, init (V89.2XXA)**
15. **Acute post-traumatic headache, intractable (G44.311)**

PLAN

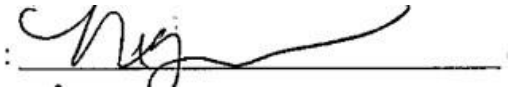
Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of diversified technique was provided specifically to OCC; C1; C2; C5; C6; C7; T1; T4; T5; T6; T7; T10; T11; T12; L4; L5; Sacrum; SI, to reduce pain reported at initial encounter by 25%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to Neck, to reduce pain reported at initial encounter by 25%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Back, to reduce pain reported at initial encounter by 25%.
- Ice/Heat (97010) consisting of ice pack, hot pack was provided specifically to Neck, Back,

to reduce pain reported at initial encounter by 25%.

- **Therapeutic Exercise (Ea. 15 Min) (97110) consisting of lumbar active therapeutic movements was provided specifically to Neck, Back for 10 minutes (1 unit), to reduce pain reported at initial encounter by 25%.**

Patient received care without incident.



(Electronic Signature)
Provider Name: Marcia Smith, DC
Date: 06/04/2024 11:39 AM

Iceberg Health Morrow
1544 Southlake Pkwy
Unit 9F
Morrow, GA 30260-0287
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Fatai Okin
Date of Birth: 4/10/1980

Provider: Angela Alvarez, DC
Date of Service: 6/5/2024

SUBJECTIVE

PRIMARY COMPLAINT
Low back, Mid back

- Burning, Aching, Sharp pain: 9/10
  Radiates to (anterior) **left upper leg (thigh)**, **right upper leg (thigh)** and **right lower leg**.
  Described as constant (up to 100% of day).

SECOND COMPLAINT
Neck, Upper back

- Throbbing, Sharp pain: 8/10
  Radiates to (posterior) left shoulder and right shoulder.

THIRD COMPLAINT
Left chest, Right chest

- Aching pain: 6/10
  Described as constant (up to 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL).

FOURTH COMPLAINT
Head

- Aching, Throbbing pain: 4/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate intensity (requires significant or complete modification to ADL).

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: OCC; C1; C2; C5; C6; C7; T1; T4; T5; T6; T7; T10; T11; T12; L4; L5; Sacrum; SI**.**

Tenderness

Left Side

- Cervical Spine (Moderate-Severe)
- Upper Trapezius (Moderate-Severe)

Right Side

- Cervical Spine (Moderate-Severe)
- Upper Trapezius (Moderate-Severe)

- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)

- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Moderate-Severe)
- Upper Trapezius (Moderate-Severe)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)

Right Side

- Cervical Spine (Moderate-Severe)
- Upper Trapezius (Moderate-Severe)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Radiculopathy, cervical region (M54.12)
2. Radiculopathy, lumbosacral region (M54.17)
3. Segmental and somatic dysfunction of lumbar region (M99.03)
4. Segmental and somatic dysfunction of cervical region (M99.01)
5. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
6. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
7. Disorder of ligament, vertebrae (M24.28)
8. Strain of muscle, fascia and tendon at neck level, init (S16.1XXA)
9. Sleep deprivation (Z72.820)
10. Strain of muscle, fascia and tendon of lower back, init (S39.012A)
11. Cervicalgia (M54.2)
12. Other muscle spasm (M62.838)
13. Segmental and somatic dysfunction of thoracic region (M99.02)
14. Person injured in unsp motor-vehicle accident, traffic, init (V89.2XXA)
15. Acute post-traumatic headache, intractable (G44.311)

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of diversified technique was provided specifically to OCC; C1; C2; C5; C6; C7; T1; T4; T5; T6; T7; T10; T11; T12; L4; L5; Sacrum; SI, to reduce pain reported at initial encounter by 25%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to Neck, **Back**, to reduce pain reported at initial encounter by 25%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Back, to reduce pain reported at initial encounter by 25%.
- Ice/Heat (97010) consisting of ice pack was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.
- **Diagnostic Tests: X-ray cervical 4-5 views (72050) was provided due to significant traumatic mechanism of injury.**

- **Diagnostic Tests: X-ray thoracic 2 views (72070) was provided due to significant traumatic mechanism of injury.**
- **Diagnostic Tests: X-ray lumbosacral 2-3 views (72100) was provided due to significant traumatic mechanism of injury.**

Attended by Dr. Marcia Smith, DC (6/5/24)

Patient received care without incident.

(Electronic Signature)
Provider Name: Angela Alvarez, DC
Date: 06/05/2024 03:41 PM

Iceberg Health Morrow
1544 Southlake Pkwy
Unit 9F
Morrow, GA 30260-0287
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Fatai Okin
Date of Birth: 4/10/1980

Provider: Marcia Smith, DC
Date of Service: 6/6/2024

SUBJECTIVE

PRIMARY COMPLAINT
Low back, Mid back

- Burning, Aching, Sharp pain: **From 9 to 8/10 (10% decrease)**
  Radiates to (anterior) **left upper leg (thigh)**, **right upper leg (thigh)** and **right lower leg**.
  Described as constant (up to 100% of day).

SECOND COMPLAINT
Neck, Upper back

- Throbbing, Sharp pain: **From 8 to 7/10 (10% decrease)**
  Radiates to (posterior) left shoulder and right shoulder.

THIRD COMPLAINT
Left chest, Right chest

- Aching pain: **From 6 to 4/10 (20% decrease)**
  Described as constant (up to 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL).

FOURTH COMPLAINT
Head

- Aching, Throbbing pain: **From 4 to 2/10 (20% decrease)**
  Described as frequent to constant (up to between 75% and 100% of day) of moderate intensity (requires significant or complete modification to ADL).

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: OCC; C1; C2; C5; C6; C7; T1; T4; T5; T6; T7; T10; T11; T12; L4; L5; Sacrum; SI**.**

Tenderness

Left Side

- Cervical Spine (Moderate-Severe)
- Upper Trapezius (Moderate-Severe)

Right Side

- Cervical Spine (Moderate-Severe)
- Upper Trapezius (Moderate-Severe)

- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)

- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Moderate-Severe)
- Upper Trapezius (Moderate-Severe)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)

Right Side

- Cervical Spine (Moderate-Severe)
- Upper Trapezius (Moderate-Severe)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Radiculopathy, cervical region (M54.12)
2. Radiculopathy, lumbosacral region (M54.17)
3. Segmental and somatic dysfunction of lumbar region (M99.03)
4. Segmental and somatic dysfunction of cervical region (M99.01)
5. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
6. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
7. Disorder of ligament, vertebrae (M24.28)
8. Strain of muscle, fascia and tendon at neck level, init (S16.1XXA)
9. Sleep deprivation (Z72.820)
10. Strain of muscle, fascia and tendon of lower back, init (S39.012A)
11. Cervicalgia (M54.2)
12. Other muscle spasm (M62.838)
13. Segmental and somatic dysfunction of thoracic region (M99.02)
14. Person injured in unsp motor-vehicle accident, traffic, init (V89.2XXA)
15. Acute post-traumatic headache, intractable (G44.311)

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of diversified technique was provided specifically to OCC; C1; C2; C5; C6; C7; T1; T4; T5; T6; T7; T10; T11; T12; L4; L5; Sacrum; SI, to reduce pain reported at initial encounter by 25%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Back, to reduce pain reported at initial encounter by 25%.
- Ice/Heat (97010) consisting of ice pack was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.

Patient received care without incident.

:

(Electronic Signature)
Provider Name: Marcia Smith, DC
Date: 06/06/2024 12:21 PM

Iceberg Health Morrow
1544 Southlake Pkwy
Unit 9F
Morrow, GA 30260-0287
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Fatai Okin
Date of Birth: 4/10/1980

Provider: Marcia Smith, DC
Date of Service: 6/11/2024

SUBJECTIVE

PRIMARY COMPLAINT
Low back, Mid back

- Burning, Aching, Sharp pain: 8/10
  Radiates to (anterior) **left upper leg (thigh)**, **right upper leg (thigh)** and **right lower leg**.
  Described as constant (up to 100% of day).

SECOND COMPLAINT
Neck, Upper back

- Throbbing, Sharp pain: 7/10
  Radiates to (posterior) left shoulder and right shoulder.

THIRD COMPLAINT
Left chest, Right chest

- Aching pain: 4/10
  Described as constant (up to 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL).

FOURTH COMPLAINT
Head

- Aching, Throbbing pain: 2/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate intensity (requires significant or complete modification to ADL).

Patient reports having MRI done Sunday. Patient reports that when at rest and not doing much his back pain is a 3/10VAS but when he is lifting, stretching or rotating pain is stated to be a 7/10VAS. Sternum pain (chest) is also triggered with neck rotation.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: OCC; C1; C2; C5; C6; C7; T1; T4; T5; T6; T7; T10; T11; T12; L4; L5; Sacrum; SI**.**

Tenderness

Left Side	Right Side

- Cervical Spine (Moderate-Severe)
- Upper Trapezius (Moderate-Severe)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)

- Cervical Spine (Moderate-Severe)
- Upper Trapezius (Moderate-Severe)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Moderate-Severe)
- Upper Trapezius (Moderate-Severe)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)

Right Side

- Cervical Spine (Moderate-Severe)
- Upper Trapezius (Moderate-Severe)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Radiculopathy, cervical region (M54.12)
2. Radiculopathy, lumbosacral region (M54.17)
3. Segmental and somatic dysfunction of lumbar region (M99.03)
4. Segmental and somatic dysfunction of cervical region (M99.01)
5. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
6. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
7. Disorder of ligament, vertebrae (M24.28)
8. Strain of muscle, fascia and tendon at neck level, init (S16.1XXA)
9. Sleep deprivation (Z72.820)
10. Strain of muscle, fascia and tendon of lower back, init (S39.012A)
11. Cervicalgia (M54.2)
12. Other muscle spasm (M62.838)
13. Segmental and somatic dysfunction of thoracic region (M99.02)
14. Person injured in unsp motor-vehicle accident, traffic, init (V89.2XXA)
15. Acute post-traumatic headache, intractable (G44.311)

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of diversified technique was provided specifically to OCC; C1; C2; C5; C6; C7; T1; T4; T5; T6; T7; T10; T11; T12; L4; L5; Sacrum; SI, to reduce pain reported at initial encounter by 25%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Back, to reduce pain reported at initial encounter by 25%.
- Ice/Heat (97010) consisting of ice pack was provided specifically to Neck, Back, to

reduce pain reported at initial encounter by 25%.

Given icing instructions for home care.

Patient received care without incident.

(Electronic Signature)
Provider Name: Marcia Smith, DC
Date: 06/11/2024 12:43 PM

Iceberg Health Morrow
1544 Southlake Pkwy
Unit 9F
Morrow, GA 30260-0287
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Fatai Okin
Date of Birth: 4/10/1980

Provider: Marcia Smith, DC
Date of Service: 6/13/2024

SUBJECTIVE

PRIMARY COMPLAINT
Low back, Mid back

- Burning, Aching, Sharp pain: **From 8 to 4/10 (40% decrease)**
  Radiates to (anterior) **left upper leg (thigh)**, **right upper leg (thigh)** and **right lower leg**. Described as **frequent (up to 75% of day)** of **moderate intensity (requires significant or complete modification to ADL)**.

SECOND COMPLAINT
Neck, Upper back

- Throbbing, Sharp pain: **From 7 to 5/10 (20% decrease)**
  Radiates to (posterior) left shoulder and right shoulder. Described as **frequent (up to 75% of day)** of **moderate intensity (requires significant or complete modification to ADL)**.

THIRD COMPLAINT
Left chest, Right chest

- Aching pain: 4/10
  Described as constant (up to 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL).

FOURTH COMPLAINT
Head

- Aching, Throbbing pain: 2/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate intensity (requires significant or complete modification to ADL).

Chest pain still remains and he **feels** it **mostly now** when **he lifts something heavy or moves in certain positions**. **Low** back pain **in the morning** when he **wakes up but as he goes through out the day** it gets **better**.

OBJECTIVE

PALPATORY FINDINGS
Spinal asymmetry/fixation: C2; C6; C7; T1; T4; T5; T6; T7; T11; T12; L4; L5; Sacrum; SI**. No findings: OCC; C1; C5; T10**.

Tenderness

Left Side

- Cervical Spine (Moderate-Severe)
- Upper Trapezius (Moderate-Severe)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)

Right Side

- Cervical Spine (Moderate-Severe)
- Upper Trapezius (Moderate-Severe)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Moderate-Severe)
- Upper Trapezius (Moderate-Severe)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)

Right Side

- Cervical Spine (Moderate-Severe)
- Upper Trapezius (Moderate-Severe)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Radiculopathy, cervical region (M54.12)
2. Radiculopathy, lumbosacral region (M54.17)
3. Segmental and somatic dysfunction of lumbar region (M99.03)
4. Segmental and somatic dysfunction of cervical region (M99.01)
5. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
6. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
7. Disorder of ligament, vertebrae (M24.28)
8. Strain of muscle, fascia and tendon at neck level, init (S16.1XXA)
9. Sleep deprivation (Z72.820)
10. Strain of muscle, fascia and tendon of lower back, init (S39.012A)
11. Cervicalgia (M54.2)
12. Other muscle spasm (M62.838)
13. Segmental and somatic dysfunction of thoracic region (M99.02)
14. Person injured in unsp motor-vehicle accident, traffic, init (V89.2XXA)
15. Acute post-traumatic headache, intractable (G44.311)

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of diversified technique was provided specifically to C2; C6; C7; T1; T4; T5; T6; T7; T11; T12; L4; L5; Sacrum; SI, to reduce pain reported at initial encounter by 25%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided

specifically to Back, to reduce pain reported at initial encounter by 25%.

- Ice/Heat (97010) consisting of ice pack was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.

Reviewed patient's MRI report and patient has a ALAR ligament injury. **Edema is seen between the Clivus and the Odontoid process**. **Referred to Ortho**.

Patient received care without incident.

(Electronic Signature)
Provider Name: Marcia Smith, DC
Date: 06/13/2024 12:00 PM

Iceberg Health Morrow
1544 Southlake Pkwy
Unit 9F
Morrow, GA 30260-0287
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Fatai Okin
Date of Birth: 4/10/1980

Provider: Marcia Smith, DC
Date of Service: 6/18/2024

SUBJECTIVE

PRIMARY COMPLAINT
Low back, Mid back

- Burning, Aching, Sharp pain: 4/10
  Radiates to (anterior) **left upper leg (thigh)**, **right upper leg (thigh)** and **right lower leg**. Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL).

SECOND COMPLAINT
Neck, Upper back

- Throbbing, Sharp pain: 5/10
  Radiates to (posterior) left shoulder and right shoulder. Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL).

THIRD COMPLAINT
Left chest, Right chest

- Aching pain: 4/10
  Described as constant (up to 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL).

FOURTH COMPLAINT
Head

- Aching, Throbbing pain: 2/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate intensity (requires significant or complete modification to ADL).

Reports increase in pain **today after trying to return to his workout regimen**. **He also reports having anxiety** and **sleepless** since the **accident**.

OBJECTIVE

PALPATORY FINDINGS
Spinal asymmetry/fixation: C2; C6; C7; T1; T4; T5; T6; T7; T11; T12; L4; L5; Sacrum; SI**.**
Tenderness

Left Side | Right Side

- Cervical Spine (Moderate-Severe)
- Upper Trapezius (Moderate-Severe)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)

- Cervical Spine (Moderate-Severe)
- Upper Trapezius (Moderate-Severe)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Moderate-Severe)
- Upper Trapezius (Moderate-Severe)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)

Right Side

- Cervical Spine (Moderate-Severe)
- Upper Trapezius (Moderate-Severe)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Radiculopathy, cervical region (M54.12)
2. Radiculopathy, lumbosacral region (M54.17)
3. Segmental and somatic dysfunction of lumbar region (M99.03)
4. Segmental and somatic dysfunction of cervical region (M99.01)
5. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
6. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
7. Disorder of ligament, vertebrae (M24.28)
8. Strain of muscle, fascia and tendon at neck level, init (S16.1XXA)
9. Sleep deprivation (Z72.820)
10. Strain of muscle, fascia and tendon of lower back, init (S39.012A)
11. Cervicalgia (M54.2)
12. Other muscle spasm (M62.838)
13. Segmental and somatic dysfunction of thoracic region (M99.02)
14. Person injured in unsp motor-vehicle accident, traffic, init (V89.2XXA)
15. Acute post-traumatic headache, intractable (G44.311)

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of diversified technique was provided specifically to C2; C6; C7; T1; T4; T5; T6; T7; T11; T12; L4; L5; Sacrum; SI, to reduce pain reported at initial encounter by 25%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Back, to reduce pain reported at initial encounter by 25%.
- Ice/Heat (97010) consisting of ice pack was provided specifically to Neck, Back, to

reduce pain reported at initial encounter by 25%.

Reports wanting to wait to see **how** he **progresses before being referred for anxiety**.

Patient received care without incident.

(Electronic Signature)
Provider Name: Marcia Smith, DC
Date: 06/18/2024 11:46 AM

Iceberg Health Morrow
1544 Southlake Pkwy
Unit 9F
Morrow, GA 30260-0287
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Fatai Okin
Date of Birth: 4/10/1980

Provider: Marcia Smith, DC
Date of Service: 6/20/2024

SUBJECTIVE

PRIMARY COMPLAINT
Low back, Mid back

- Burning, Aching, Sharp pain: 4/10
  Radiates to (anterior) **left upper leg (thigh)**, **right upper leg (thigh)** and **right lower leg**. Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL).

SECOND COMPLAINT
Neck, Upper back

- Throbbing, Sharp pain: **From 5 to 3/10 (20% decrease)**
  Radiates to (posterior) left shoulder and right shoulder. Described as **intermittent (up to 50% of day)** of **slight to moderate intensity**.

THIRD COMPLAINT
Left chest, Right chest

- Aching pain: 4/10
  Described as **frequent (up to 75% of day)** of **moderate intensity (requires significant or complete modification to ADL)**.

FOURTH COMPLAINT
Head

- Aching, Throbbing pain: 2/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate intensity (requires significant or complete modification to ADL).

C/o LBP and chest pain

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C2; C6; C7; T1; T5; T6; T7; L4; L5; Sacrum; SI**. No findings: T4; T11; T12**.

Tenderness

Left Side	Right Side

- Cervical Spine (Moderate-Severe)
- Upper Trapezius (Moderate-Severe)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)

- Cervical Spine (Moderate-Severe)
- Upper Trapezius (Moderate-Severe)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Moderate-Severe)
- Upper Trapezius (Moderate-Severe)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)

Right Side

- Cervical Spine (Moderate-Severe)
- Upper Trapezius (Moderate-Severe)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Radiculopathy, cervical region (M54.12)
2. Radiculopathy, lumbosacral region (M54.17)
3. Segmental and somatic dysfunction of lumbar region (M99.03)
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7. Disorder of ligament, vertebrae (M24.28)
8. Strain of muscle, fascia and tendon at neck level, init (S16.1XXA)
9. Sleep deprivation (Z72.820)
10. Strain of muscle, fascia and tendon of lower back, init (S39.012A)
11. Cervicalgia (M54.2)
12. Other muscle spasm (M62.838)
13. Segmental and somatic dysfunction of thoracic region (M99.02)
14. Person injured in unsp motor-vehicle accident, traffic, init (V89.2XXA)
15. Acute post-traumatic headache, intractable (G44.311)

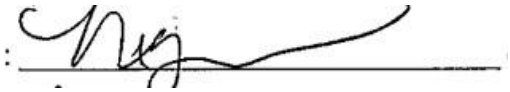
PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of diversified technique was provided specifically to C6; C7; T1; T5; T6; T7; L4; L5; Sacrum; SI, to reduce pain reported at initial encounter by 25%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Back, to reduce pain reported at initial encounter by 25%.
- Ice/Heat (97010) consisting of ice pack was provided specifically to Neck, Back, to

reduce pain reported at initial encounter by 25%.

Patient received care without incident.



(Electronic Signature)
Provider Name: Marcia Smith, DC
Date: 06/20/2024 05:50 PM

Iceberg Health Morrow
1544 Southlake Pkwy
Unit 9F
Morrow, GA 30260-0287
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Fatai Okin
Date of Birth: 4/10/1980

Provider: Marcia Smith, DC
Date of Service: 6/25/2024

SUBJECTIVE

PRIMARY COMPLAINT
Low back, Mid back

- Burning, Aching, Sharp pain: 4/10
  Radiates to (anterior) **left upper leg (thigh)**, **right upper leg (thigh)** and **right lower leg**. Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL).

SECOND COMPLAINT
Neck, Upper back

- Throbbing, Sharp pain: 3/10
  Radiates to (posterior) left shoulder and right shoulder. Described as intermittent (up to 50% of day) of slight to moderate intensity.

THIRD COMPLAINT
Left chest, Right chest

- Aching pain: 4/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL).

FOURTH COMPLAINT
Head

- Aching, Throbbing pain: 2/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate intensity (requires significant or complete modification to ADL).

OBJECTIVE

PALPATORY FINDINGS
Spinal asymmetry/fixation: C2; C6; C7; T1; T5; T6; T7; L4; L5; Sacrum; SI**.**
Tenderness

Left Side

- Cervical Spine (Moderate-Severe)
- Upper Trapezius

Right Side

- Cervical Spine (Moderate-Severe)
- Upper Trapezius

(Moderate-Severe)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)

(Moderate-Severe)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)

Spasm/Hypertonicity

Left Side
- Cervical Spine (Moderate-Severe)
- Upper Trapezius (Moderate-Severe)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)

Right Side
- Cervical Spine (Moderate-Severe)
- Upper Trapezius (Moderate-Severe)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Radiculopathy, cervical region (M54.12)
2. Radiculopathy, lumbosacral region (M54.17)
3. Segmental and somatic dysfunction of lumbar region (M99.03)
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5. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
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11. Cervicalgia (M54.2)
12. Other muscle spasm (M62.838)
13. Segmental and somatic dysfunction of thoracic region (M99.02)
14. Person injured in unsp motor-vehicle accident, traffic, init (V89.2XXA)
15. Acute post-traumatic headache, intractable (G44.311)

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of diversified technique was provided specifically to **C2**; C6; C7; T1; T5; T6; T7; L4; L5; Sacrum; SI, to reduce pain reported at initial encounter by 25%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Back, to reduce pain reported at initial encounter by 25%.
- Ice/Heat (97010) consisting of ice pack was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.

Patient received care without incident.

:

(Electronic Signature)
Provider Name: Marcia Smith, DC
Date: 06/25/2024 02:55 PM

Iceberg Health Morrow
1544 Southlake Pkwy
Unit 9F
Morrow, GA 30260-0287
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Exam

Patient Name: Fatai Okin
Date of Birth: 4/10/1980

Provider: Marcia Smith, DC
Date of Service: 6/27/2024

SUBJECTIVE

PRIMARY COMPLAINT
Low back, Mid back

- Burning, Aching, Sharp pain: **From 4 to 10/10 (60% increase)**
  Radiates to (anterior) **left upper leg (thigh)**, **right upper leg (thigh)** and **right lower leg**. Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL).

SECOND COMPLAINT
Neck, Upper back

- Throbbing, Sharp pain: 3/10
  Radiates to (posterior) left shoulder and right shoulder. Described as intermittent (up to 50% of day) of slight to moderate intensity.

THIRD COMPLAINT
Left chest, Right chest

- Aching pain: 4/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL).

FOURTH COMPLAINT
Head

- Aching, Throbbing pain: 2/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate intensity (requires significant or complete modification to ADL).

Continues to c/o chest pain

OBJECTIVE

PALPATORY FINDINGS
Spinal asymmetry/fixation: C2; C6; C7; T1; T5; T6; T7; L4; L5; Sacrum; SI**.**
Tenderness

Left Side

- Cervical Spine **(decreased 20-35%**

Right Side

- Cervical Spine **(decreased**

to mild)

- Upper Trapezius **(decreased 20-35% to mild)**
- Thoracic Spine **(decreased 20-35% to mild)**
- Lower Back **(decreased 20-35% to mild)**
- **Pectoralis (mild-moderate)**

20-35% to mild)

- Upper Trapezius **(decreased 20-35% to mild)**
- Thoracic Spine **(decreased 20-35% to mild)**
- Lower Back **(decreased 20-35% to mild)**
- **Pectoralis**

Spasm/Hypertonicity

Left Side

- Cervical Spine **(decreased 20-35% to mild)**
- Upper Trapezius **(decreased 20-35% to mild)**
- Thoracic Spine **(decreased 20-35% to mild)**
- Lower Back **(decreased 10-20% to mild-moderate)**

Right Side

- Cervical Spine **(decreased 20-35% to mild)**
- Upper Trapezius **(decreased 20-35% to mild)**
- Thoracic Spine **(decreased 20-35% to mild)**
- Lower Back **(decreased 10-20% to mild-moderate)**

GAIT

Ambulation: Normal.
Gait: Normal.

ORTHOPEDIC AND NEUROLOGICAL TESTS

Test	Left	Right	Comment
Shoulder depression test[1]	Positive	Positive	
Milgram's test[2]	Positive	Positive	
Straight leg raise[3]	**WNL**	Positive	

[1]Pain in the arm could indicate adhesions of the nerve roots. Local pain could indicate sprain or strain or adhesions of the shoulder joint capsule.

[2]Lower back pain may indicate a herniated disc.

[3]Perhipheral pain is suggestive of nerve root irritation, such as that due to a herniated disc.

Foraminal compression test, **Maximal foraminal compression test**, **Spinal percussion test-thoracic**, **Kemp's test**, Apley scratch test and **Patrick test** are within normal limits.

REFLEXES

Location	Left	Right	Comment
Patellar	1/4	1/4	

0-Absent; 1-Diminished; 2-Normal; 3-Exaggerated; 4-Hyper-Reflexive.

Biceps and **Brachioradialis** are within normal limits bilaterally.

RANGE OF MOTION

Cervical ROM - estimated

Test	Measurement	Limited By	Comment
Flexion	**45° (90% of normal, increased by 80%)**	Pain	
Extension	**45° (75% of normal, increased by 50%)**	Pain	
Left lateral flexion	**40° (88% of normal, increased by 33%)**	Pain	
Right lateral flexion	**40° (88% of normal, increased by 60%)**	Pain	
Left rotation	**65° (81% of normal, increased by 18%)**	Pain	
Right rotation	**65° (81% of normal, increased by 18%)**	Pain	

Lumbar ROM - estimated

Test	Measurement	Limited By	Comment
Flexion	**40° (66% of normal, increased by 166%)**	Pain	
Extension	**20° (80% of normal, increased by 300%)**	Pain	
Left lateral flexion	**20° (80% of normal, increased by 100%)**	Pain	
Right ateral flexion	**20° (80% of normal, increased by 100%)**	Pain	
Left rotation	20° (66% of normal)		

Right rotation within normal limits.

MUSCLE STRENGTH

Upper Muscle Strength

Shoulder abductors, Shoulder adductors, Shoulder flexors, Shoulder extensors, Elbow flexors and Elbow extensors are within normal limits bilaterally.

Lower Muscle Strength

Location	Left	Right	Comment
Hip extensors	4/5	WNL	
Knee extensors	4/5	4/5	
Knee flexors	4/5	4/5	

0-No-muscle-activity; 1-Flicker-of-movement; 2-Movement-with-gravity-eliminated; 3-Movement-against-gravity-only; 4-Movement-against-gravity-with-some-resistance; 5-Movement-against-gravity-with-full-resistance.

Hip flexors within normal limits bilaterally.

DERMATOMES

Anterior

Left-Cervical, Right-Cervical, Left-Lumbar and Right-Lumbar are within normal limits.

ASSESSMENT

Stage of Care: Rehabilitative

DIAGNOSES

1. Radiculopathy, cervical region (M54.12)
2. Radiculopathy, lumbosacral region (M54.17)
3. Segmental and somatic dysfunction of lumbar region (M99.03)
4. Segmental and somatic dysfunction of cervical region (M99.01)
5. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
6. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
7. Disorder of ligament, vertebrae (M24.28)
8. Strain of muscle, fascia and tendon at neck level, init (S16.1XXA)
9. Sleep deprivation (Z72.820)
10. Strain of muscle, fascia and tendon of lower back, init (S39.012A)
11. Cervicalgia (M54.2)
12. Other muscle spasm (M62.838)
13. Segmental and somatic dysfunction of thoracic region (M99.02)
14. Person injured in unsp motor-vehicle accident, traffic, init (V89.2XXA)
15. Acute post-traumatic headache, intractable (G44.311)

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of diversified technique was provided specifically to C2; C6; C7; T1; T5; T6; T7; L4; L5; Sacrum; SI, to reduce pain reported at initial encounter by 25%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Back, to reduce pain reported at initial encounter by 25%.
- Ice/Heat (97010) consisting of ice pack was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.
- **Established Patient evaluation and management service, 99213, was provided today.**

Patient received care without incident.

(Electronic Signature)
Provider Name: Marcia Smith, DC
Date: 06/27/2024 12:35 PM

Addenda

Patient BP: 124/77 P:61

Electronic Signature: Smith, Marcia DC

Added on Jun 27, 2024

Iceberg Health Morrow
1544 Southlake Pkwy
Unit 9F
Morrow, GA 30260-0287
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Fatai Okin
Date of Birth: 4/10/1980

Provider: Marcia Smith, DC
Date of Service: 8/6/2024

SUBJECTIVE

PRIMARY COMPLAINT
Low back, Mid back

- Burning, Aching, Sharp pain: **From 10 to 4/10 (60% decrease)**
  Radiates to (anterior) **left upper leg (thigh)**, **right upper leg (thigh)** and **right lower leg**.
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL).

SECOND COMPLAINT
Neck, Upper back

- Throbbing, Sharp pain
  Radiates to (posterior) left shoulder and right shoulder. Described as of **minimal intensity (requires very little or no modification to ADL)**.

THIRD COMPLAINT
Left chest, Right chest

- Aching pain: **From 4 to 2/10 (20% decrease)**
  Described as **infrequent** of **minimal to slight intensity (symptom is not alleviated by any amount of modification to ADL)**.

FOURTH COMPLAINT
Head

- Aching, Throbbing pain: 2/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate intensity (requires significant or complete modification to ADL).

OBJECTIVE

PALPATORY FINDINGS
Spinal asymmetry/fixation: C2; C6; C7; T1; T5; T6; T7; L4; L5; Sacrum; SI**.**
Tenderness

Left Side

- Cervical Spine (Mild)
- Upper Trapezius (Mild)

Right Side

- Cervical Spine (Mild)
- Upper Trapezius (Mild)

- Thoracic Spine (Mild)
- Lower Back (Mild)
- Pectoralis (Mild-Moderate)

- Thoracic Spine (Mild)
- Lower Back (Mild)
- Pectoralis

Spasm/Hypertonicity

Left Side

- Cervical Spine (Mild)
- Upper Trapezius (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild-Moderate)

Right Side

- Cervical Spine (Mild)
- Upper Trapezius (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild-Moderate)

ASSESSMENT

Stage of Care: Rehabilitative

DIAGNOSES

1. Radiculopathy, cervical region (M54.12)
2. Radiculopathy, lumbosacral region (M54.17)
3. Segmental and somatic dysfunction of lumbar region (M99.03)
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14. Person injured in unsp motor-vehicle accident, traffic, init (V89.2XXA)
15. Acute post-traumatic headache, intractable (G44.311)

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of diversified technique was provided specifically to C2; C6; C7; T1; T5; T6; T7; L4; L5; Sacrum; SI, to reduce pain reported at initial encounter by 25%.
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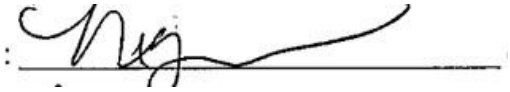
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Back, to reduce pain reported at initial encounter by 25%.
- Ice/Heat (97010) consisting of ice pack was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.
- **Therapeutic Exercise (Ea. 15 Min) (97110) consisting of lumbar active therapeutic movements was provided specifically to Neck, Back for 10 minutes (1 unit), to reduce pain reported at initial encounter by 25%.**

Patient received care without incident.



(Electronic Signature)
Provider Name: Marcia Smith, DC
Date: 08/06/2024 04:10 PM

Iceberg Health Morrow
1544 Southlake Pkwy
Unit 9F
Morrow, GA 30260-0287
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Fatai Okin
Date of Birth: 4/10/1980

Provider: Marcia Smith, DC
Date of Service: 8/8/2024

SUBJECTIVE

PRIMARY COMPLAINT
Low back, Mid back

- Burning, Aching, Sharp pain: 4/10
  Radiates to (anterior) **left upper leg (thigh)**, **right upper leg (thigh)** and **right lower leg**. Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL).

SECOND COMPLAINT
Left chest, Right chest

- Aching pain: 2/10
  Described as infrequent of minimal to slight intensity (symptom is not alleviated by any amount of modification to ADL).

THIRD COMPLAINT
Head

- Aching, Throbbing pain: 2/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate intensity (requires significant or complete modification to ADL).

FOURTH COMPLAINT
Neck, Upper back

- Throbbing, Sharp pain
  Radiates to (posterior) left shoulder and right shoulder. Described as minimal intensity (requires very little or no modification to ADL).

OBJECTIVE

PALPATORY FINDINGS
Spinal asymmetry/fixation: C2; C6; C7; T1; T5; T6; T7; L4; L5; Sacrum; SI**.**
Tenderness

Left Side

- Cervical Spine (Mild)
- Upper Trapezius (Mild)

Right Side

- Cervical Spine (Mild)
- Upper Trapezius (Mild)

- Thoracic Spine (Mild)
- Lower Back (Mild)
- Pectoralis (Mild-Moderate)

- Thoracic Spine (Mild)
- Lower Back (Mild)
- Pectoralis

Spasm/Hypertonicity

Left Side

- Cervical Spine (Mild)
- Upper Trapezius (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild-Moderate)

Right Side

- Cervical Spine (Mild)
- Upper Trapezius (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild-Moderate)

ASSESSMENT

Stage of Care: Rehabilitative

DIAGNOSES

1. Radiculopathy, cervical region (M54.12)
2. Radiculopathy, lumbosacral region (M54.17)
3. Segmental and somatic dysfunction of lumbar region (M99.03)
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14. Person injured in unsp motor-vehicle accident, traffic, init (V89.2XXA)
15. Acute post-traumatic headache, intractable (G44.311)

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of diversified technique was provided specifically to C2; C6; C7; T1; T5; T6; T7; L4; L5; Sacrum; SI, to reduce pain reported at initial encounter by 25%.
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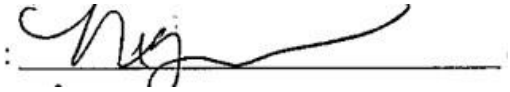
- Therapeutic Exercise (Ea. 15 Min) (97110) consisting of lumbar active therapeutic movements was provided specifically to Neck, Back for 10 minutes (1 unit), to reduce pain reported at initial encounter by 25%.

Patient received care without incident.



(Electronic Signature)
Provider Name: Marcia Smith, DC
Date: 08/08/2024 04:14 PM

Iceberg Health Morrow
1544 Southlake Pkwy
Unit 9F
Morrow, GA 30260-0287
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Fatai Okin
Date of Birth: 4/10/1980

Provider: Marcia Smith, DC
Date of Service: 8/15/2024

SUBJECTIVE

PRIMARY COMPLAINT
Low back, Mid back

- Burning, Aching, Sharp pain: **From 4 to 3/10 (10% decrease)**
  Radiates to (anterior) **left upper leg (thigh)**, **right upper leg (thigh)** and **right lower leg**. Described as **occasional to intermittent (up to between 25% and 50% of day)** of **slight intensity (requires some modification to ADL)**.

SECOND COMPLAINT
Left chest, Right chest

- Aching pain: 2/10
  Described as infrequent of minimal to slight intensity (symptom is not alleviated by any amount of modification to ADL).

THIRD COMPLAINT
Head

- Aching, Throbbing pain: 2/10
  Described as **occasional to intermittent (up to between 25% and 50% of day)** of moderate intensity (requires significant or complete modification to ADL).

FOURTH COMPLAINT
Neck, Upper back

- Throbbing, Sharp pain: **2/10**
  Radiates to (posterior) left shoulder and right shoulder. Described as **occasional to intermittent (up to between 25% and 50% of day)** of **minimal to slight intensity (symptom is not alleviated by any amount of modification to ADL)**.

OBJECTIVE

PALPATORY FINDINGS
Spinal asymmetry/fixation: C2; C6; C7; T1; T5; T6; T7; L4; L5; Sacrum; SI**.**
Tenderness

Left Side

- Cervical Spine (Mild)

Right Side

- Cervical Spine (Mild)

- Upper Trapezius (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)
- Pectoralis (Mild-Moderate)

- Upper Trapezius (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)
- Pectoralis

Spasm/Hypertonicity

Left Side

- Cervical Spine (Mild)
- Upper Trapezius (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild-Moderate)

Right Side

- Cervical Spine (Mild)
- Upper Trapezius (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild-Moderate)

ASSESSMENT

Stage of Care: Rehabilitative

DIAGNOSES

1. Radiculopathy, cervical region (M54.12)
2. Radiculopathy, lumbosacral region (M54.17)
3. Segmental and somatic dysfunction of lumbar region (M99.03)
4. Segmental and somatic dysfunction of cervical region (M99.01)
5. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
6. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
7. Disorder of ligament, vertebrae (M24.28)
8. Strain of muscle, fascia and tendon at neck level, init (S16.1XXA)
9. Sleep deprivation (Z72.820)
10. Strain of muscle, fascia and tendon of lower back, init (S39.012A)
11. Cervicalgia (M54.2)
12. Other muscle spasm (M62.838)
13. Segmental and somatic dysfunction of thoracic region (M99.02)
14. Person injured in unsp motor-vehicle accident, traffic, init (V89.2XXA)
15. Acute post-traumatic headache, intractable (G44.311)

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of diversified technique was provided specifically to C2; C6; C7; T1; T5; T6; T7; L4; L5; Sacrum; SI, to reduce pain reported at initial encounter by 25%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Back, to reduce pain reported at initial encounter by 25%.
- Ice/Heat (97010) consisting of ice pack was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.
- Therapeutic Exercise (Ea. 15 Min) (97110) consisting of lumbar active therapeutic movements was provided specifically to Neck, Back for 10 minutes (1 unit), to reduce pain reported at initial encounter by 25%.

Patient received care without incident.

(Electronic Signature)
Provider Name: Marcia Smith, DC
Date: 08/15/2024 04:42 PM

Iceberg Health Morrow
1544 Southlake Pkwy
Unit 9F
Morrow, GA 30260-0287
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Fatai Okin
Date of Birth: 4/10/1980

Provider: Mary Dearborn, DC
Date of Service: 8/22/2024

SUBJECTIVE

PRIMARY COMPLAINT
Low back, Mid back

- Burning, Aching, Sharp pain: **From 3 to 2/10 (10% decrease)**
  Radiates to (anterior) **left upper leg (thigh)**, **right upper leg (thigh)** and **right lower leg**. Described as occasional to intermittent (up to between 25% and 50% of day) of slight intensity (requires some modification to ADL).

SECOND COMPLAINT
Left chest, Right chest

- Aching pain: 2/10
  Described as infrequent of minimal to slight intensity (symptom is not alleviated by any amount of modification to ADL).

THIRD COMPLAINT
Head

- Aching, Throbbing pain: 2/10
  Described as occasional to intermittent (up to between 25% and 50% of day) of moderate intensity (requires significant or complete modification to ADL).

FOURTH COMPLAINT
Neck, Upper back

- Throbbing, Sharp pain: 2/10
  Radiates to (posterior) left shoulder and right shoulder. Described as occasional to intermittent (up to between 25% and 50% of day) of minimal to slight intensity (symptom is not alleviated by any amount of modification to ADL).

He mentioned he feels that he is improving

OBJECTIVE

PALPATORY FINDINGS
Spinal asymmetry/fixation: C2; C6; C7; T1; T5; T6; T7; L4; L5; Sacrum; SI**.**

Tenderness

Left Side	Right Side

- Cervical Spine (Mild)
- Upper Trapezius (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)
- Pectoralis (Mild-Moderate)

- Cervical Spine (Mild)
- Upper Trapezius (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)
- Pectoralis

Spasm/Hypertonicity

Left Side

- Cervical Spine (Mild)
- Upper Trapezius (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild-Moderate)

Right Side

- Cervical Spine (Mild)
- Upper Trapezius (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild-Moderate)

ASSESSMENT

Stage of Care: Rehabilitative

DIAGNOSES

1. Radiculopathy, cervical region (M54.12)
2. Radiculopathy, lumbosacral region (M54.17)
3. Segmental and somatic dysfunction of lumbar region (M99.03)
4. Segmental and somatic dysfunction of cervical region (M99.01)
5. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
6. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
7. Disorder of ligament, vertebrae (M24.28)
8. Strain of muscle, fascia and tendon at neck level, init (S16.1XXA)
9. Sleep deprivation (Z72.820)
10. Strain of muscle, fascia and tendon of lower back, init (S39.012A)
11. Cervicalgia (M54.2)
12. Other muscle spasm (M62.838)
13. Segmental and somatic dysfunction of thoracic region (M99.02)
14. Person injured in unsp motor-vehicle accident, traffic, init (V89.2XXA)
15. Acute post-traumatic headache, intractable (G44.311)

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of diversified technique was provided specifically to C2; C6; C7; T1; T5; T6; T7; L4; L5; Sacrum; SI, to reduce pain reported at initial encounter by 25%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Back, to reduce pain reported at initial encounter by 25%.
- Ice/Heat (97010) consisting of ice pack was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.

Patient received care without incident.

Mary Dearborn

(Electronic Signature)
Provider Name: Mary Dearborn, DC
Date: 08/22/2024 03:40 PM

Iceberg Health Morrow
1544 Southlake Pkwy
Unit 9F
Morrow, GA 30260-0287
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Fatai Okin
Date of Birth: 4/10/1980

Provider: Amanda Ruiz, DC
Date of Service: 8/27/2024

SUBJECTIVE

PRIMARY COMPLAINT
Low back, Mid back

- Burning, Aching, Sharp pain: 2/10
  Radiates to (anterior) **left upper leg (thigh)**, **right upper leg (thigh)** and **right lower leg**. Described as occasional to intermittent (up to between 25% and 50% of day) of slight intensity (requires some modification to ADL).

SECOND COMPLAINT
Left chest, Right chest

- Aching pain: **From 2 to 1/10 (10% decrease)**
  Described as infrequent of minimal to slight intensity (symptom is not alleviated by any amount of modification to ADL).

THIRD COMPLAINT
Head

- Aching, Throbbing pain: 2/10
  Described as occasional to intermittent (up to between 25% and 50% of day) of moderate intensity (requires significant or complete modification to ADL).

FOURTH COMPLAINT
Neck, Upper back

- Throbbing, Sharp pain: 2/10
  Radiates to (posterior) left shoulder and right shoulder. Described as occasional to intermittent (up to between 25% and 50% of day) of minimal to slight intensity (symptom is not alleviated by any amount of modification to ADL).

He **stated that** he feels **the same as last visit**.

OBJECTIVE

PALPATORY FINDINGS
Spinal asymmetry/fixation: C2; C6; C7; T1; T5; T6; T7; L4; L5; Sacrum; SI**.**

Tenderness

Left Side	Right Side

- Cervical Spine (Mild)
- Upper Trapezius (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)
- Pectoralis (Mild-Moderate)

- Cervical Spine (Mild)
- Upper Trapezius (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)
- Pectoralis

Spasm/Hypertonicity

Left Side

- Cervical Spine (Mild)
- Upper Trapezius (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild-Moderate)

Right Side

- Cervical Spine (Mild)
- Upper Trapezius (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild-Moderate)

ASSESSMENT

Stage of Care: Rehabilitative

DIAGNOSES

1. Radiculopathy, cervical region (M54.12)
2. Radiculopathy, lumbosacral region (M54.17)
3. Segmental and somatic dysfunction of lumbar region (M99.03)
4. Segmental and somatic dysfunction of cervical region (M99.01)
5. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
6. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
7. Disorder of ligament, vertebrae (M24.28)
8. Strain of muscle, fascia and tendon at neck level, init (S16.1XXA)
9. Sleep deprivation (Z72.820)
10. Strain of muscle, fascia and tendon of lower back, init (S39.012A)
11. Cervicalgia (M54.2)
12. Other muscle spasm (M62.838)
13. Segmental and somatic dysfunction of thoracic region (M99.02)
14. Person injured in unsp motor-vehicle accident, traffic, init (V89.2XXA)
15. Acute post-traumatic headache, intractable (G44.311)

PROGNOSIS

Good - Symptomatic and functional recovery is expected in approximately 8-12 weeks.

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of **Instrument adjusting**, diversified technique was provided specifically to C7; T1; **T7**; **T8**; L5; **Pelvis**, to reduce pain reported at initial encounter by 25%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Back, to reduce pain reported at initial encounter by 25%.
- Ice/Heat (97010) consisting of ice pack was provided specifically to Neck, Back, to

reduce pain reported at initial encounter by 25%.

Patient received care without incident.

(Electronic Signature)
Provider Name: Amanda Ruiz, DC
Date: 08/27/2024 03:21 PM

Iceberg Health Morrow
1544 Southlake Pkwy
Unit 9F
Morrow, GA 30260-0287
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Fatai Okin
Date of Birth: 4/10/1980

Provider: Mary Dearborn, DC
Date of Service: 8/30/2024

SUBJECTIVE

PRIMARY COMPLAINT
Low back, Mid back

- Burning, Aching, Sharp pain: 2/10
  Radiates to (anterior) **left upper leg (thigh)**, **right upper leg (thigh)** and **right lower leg**. Described as occasional to intermittent (up to between 25% and 50% of day) of slight intensity (requires some modification to ADL).

SECOND COMPLAINT
Left chest, Right chest

- Aching pain: 1/10
  Described as infrequent of minimal to slight intensity (symptom is not alleviated by any amount of modification to ADL).

THIRD COMPLAINT
Head

- Aching, Throbbing pain: 2/10
  Described as occasional to intermittent (up to between 25% and 50% of day) of moderate intensity (requires significant or complete modification to ADL).

FOURTH COMPLAINT
Neck, Upper back

- Throbbing, Sharp pain: 2/10
  Radiates to (posterior) left shoulder and right shoulder. Described as occasional to intermittent (up to between 25% and 50% of day) of minimal to slight intensity (symptom is not alleviated by any amount of modification to ADL).

He **is continuing to improve**, **but his pain is worse in** the **morning and gets better with light activity**.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C2; C6; C7; T1; T5; T6; T7; L4; L5; Sacrum; SI**.**

Tenderness

Left Side	Right Side

- Cervical Spine (Mild)
- Upper Trapezius (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)
- Pectoralis (Mild-Moderate)

- Cervical Spine (Mild)
- Upper Trapezius (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)
- Pectoralis

Spasm/Hypertonicity

Left Side

- Cervical Spine (Mild)
- Upper Trapezius (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild-Moderate)

Right Side

- Cervical Spine (Mild)
- Upper Trapezius (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild-Moderate)

ASSESSMENT

Stage of Care: Rehabilitative

DIAGNOSES

1. Radiculopathy, cervical region (M54.12)
2. Radiculopathy, lumbosacral region (M54.17)
3. Segmental and somatic dysfunction of lumbar region (M99.03)
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5. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
6. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
7. Disorder of ligament, vertebrae (M24.28)
8. Strain of muscle, fascia and tendon at neck level, init (S16.1XXA)
9. Sleep deprivation (Z72.820)
10. Strain of muscle, fascia and tendon of lower back, init (S39.012A)
11. Cervicalgia (M54.2)
12. Other muscle spasm (M62.838)
13. Segmental and somatic dysfunction of thoracic region (M99.02)
14. Person injured in unsp motor-vehicle accident, traffic, init (V89.2XXA)
15. Acute post-traumatic headache, intractable (G44.311)

PROGNOSIS

Good - Symptomatic and functional recovery is expected in approximately 8-12 weeks.

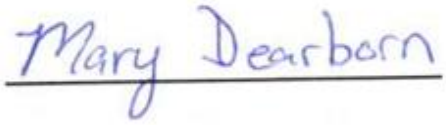
PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique was provided specifically to **C2**; **C6**; **C7**; T1; **T5**; **T6**; **T7**; **L4**; L5; **Sacrum**; **SI**, to reduce pain reported at initial encounter by 25%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Back, to reduce pain reported at initial encounter by 25%.
- Ice/Heat (97010) consisting of ice pack was provided specifically to Neck, Back, to

reduce pain reported at initial encounter by 25%.

Patient received care without incident.



(Electronic Signature)
Provider Name: Mary Dearborn, DC
Date: 08/30/2024 03:23 PM

Iceberg Health Morrow
1544 Southlake Pkwy
Unit 9F
Morrow, GA 30260-0287
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Fatai Okin
Date of Birth: 4/10/1980

Provider: Amanda Ruiz, DC
Date of Service: 9/3/2024

SUBJECTIVE

PRIMARY COMPLAINT
Low back, Mid back

- Burning, Aching, Sharp pain: 2/10
  Radiates to (anterior) **left upper leg (thigh)**, **right upper leg (thigh)** and **right lower leg**. Described as occasional to intermittent (up to between 25% and 50% of day) of slight intensity (requires some modification to ADL).

SECOND COMPLAINT
Left chest, Right chest

- Aching pain: 1/10
  Described as infrequent of minimal to slight intensity (symptom is not alleviated by any amount of modification to ADL).

THIRD COMPLAINT
Head

- Aching, Throbbing pain: 2/10
  Described as occasional to intermittent (up to between 25% and 50% of day) of moderate intensity (requires significant or complete modification to ADL).

FOURTH COMPLAINT
Neck, Upper back

- Throbbing, Sharp pain: 2/10
  Radiates to (posterior) left shoulder and right shoulder. Described as occasional to intermittent (up to between 25% and 50% of day) of minimal to slight intensity (symptom is not alleviated by any amount of modification to ADL).

OBJECTIVE

PALPATORY FINDINGS
Spinal asymmetry/fixation: C2; C6; C7; T1; T5; T6; T7; L4; L5; Sacrum; SI; **C3; C4; C5; L3; Pelvis.**
Tenderness

Left Side

- Cervical Spine (Mild)

Right Side

- Cervical Spine (Mild)

- Upper Trapezius (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)
- Pectoralis (Mild-Moderate)

- Upper Trapezius (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)
- Pectoralis

Spasm/Hypertonicity

Left Side

- Cervical Spine (Mild)
- Upper Trapezius (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild-Moderate)

Right Side

- Cervical Spine (Mild)
- Upper Trapezius (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild-Moderate)

ASSESSMENT

Stage of Care: Rehabilitative

DIAGNOSES

1. Radiculopathy, cervical region (M54.12)
2. Radiculopathy, lumbosacral region (M54.17)
3. Segmental and somatic dysfunction of lumbar region (M99.03)
4. Segmental and somatic dysfunction of cervical region (M99.01)
5. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
6. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
7. Disorder of ligament, vertebrae (M24.28)
8. Strain of muscle, fascia and tendon at neck level, init (S16.1XXA)
9. Sleep deprivation (Z72.820)
10. Strain of muscle, fascia and tendon of lower back, init (S39.012A)
11. Cervicalgia (M54.2)
12. Other muscle spasm (M62.838)
13. Segmental and somatic dysfunction of thoracic region (M99.02)
14. Person injured in unsp motor-vehicle accident, traffic, init (V89.2XXA)
15. Acute post-traumatic headache, intractable (G44.311)

PROGNOSIS

Good - Symptomatic and functional recovery is expected in approximately 8-12 weeks.

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique, **Thompson technique** was provided specifically to **C3**; **C4**; T5; T6; T7; L4; L5; **Pelvis**; SI, to reduce pain reported at initial encounter by 25%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Back, to reduce pain reported at initial encounter by 25%.
- Ice/Heat (97010) consisting of ice pack was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.

Patient received care without incident.

(Electronic Signature)
Provider Name: Amanda Ruiz, DC
Date: 09/03/2024 03:35 PM

Iceberg Health Morrow
1544 Southlake Pkwy
Unit 9F
Morrow, GA 30260-0287
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Exam

Patient Name: Fatai Okin
Date of Birth: 4/10/1980

Provider: Amanda Ruiz, DC
Date of Service: 9/5/2024

SUBJECTIVE

PRIMARY COMPLAINT
Low back, Mid back

- Burning, Aching, Sharp pain: 2/10
  Radiates to (anterior) **left upper leg (thigh)**, **right upper leg (thigh)** and **right lower leg**. Described as **occasional (up to 25% of day)** of slight intensity (requires some modification to ADL); increased by **ADL's**; decreased by **rest**.

SECOND COMPLAINT
Left chest, Right chest

- Aching pain: 1/10
  Described as infrequent of minimal to slight intensity (symptom is not alleviated by any amount of modification to ADL); increased by **ADL's**; decreased by **rest**.

THIRD COMPLAINT
Head

- Aching, Throbbing pain
  Described as **infrequent** of **minimal intensity (requires very little or no modification to ADL)**; increased by **ADL's**; decreased by **rest**.

FOURTH COMPLAINT
Neck, Upper back

- Throbbing, Sharp pain: 2/10
  Radiates to (posterior) left shoulder and right shoulder. Described as **infrequent** of **minimal intensity (requires very little or no modification to ADL)**; increased by **ADL's**; decreased by **rest**.

OBJECTIVE

VITALS
Height: 64 in Weight: 153 lbs BMI: 26.3
Pulse rate: 61 BPM (relaxed)
Blood Pressure (seated): right -N/A left-114/73 mmHg
Demeanor: coherent and relaxed.

PALPATORY FINDINGS
Spinal asymmetry/fixation: C2; C3; C4; C5; C6; C7; T1; T6; T7; L3; L4; L5; Pelvis; Sacrum; SI; **T2; T3. No findings: T5**.

Tenderness

Left Side

- Cervical Spine (Mild)
- Upper Trapezius (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)
- Pectoralis (Mild-Moderate)

Right Side

- Cervical Spine (Mild)
- Upper Trapezius (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)
- Pectoralis

Spasm/Hypertonicity

Left Side

- Cervical Spine **(decreased 5-10% to minimal)**
- Upper Trapezius **(decreased 5-10% to minimal)**
- Thoracic Spine **(decreased 5-10% to minimal)**

Right Side

- Cervical Spine **(decreased 5-10% to minimal)**
- Upper Trapezius **(decreased 5-10% to minimal)**
- Thoracic Spine **(decreased 5-10% to minimal)**
- Lower Back **(decreased 10-20% to minimal)**

Trigger Points

Left Side

- **Cervical Spine (minimal)**
- **Thoracic Spine (minimal)**
- **Lower Back (minimal)**

Right Side

- **Cervical Spine (minimal)**
- **Thoracic Spine (minimal)**
- **Lower Back (minimal)**

GAIT

Ambulation: Normal.
Gait: Normal.

ORTHOPEDIC AND NEUROLOGICAL TESTS

Foraminal compression test, **Shoulder depression test**, Maximal foraminal compression test, Spinal percussion test-thoracic, Kemp's test, **Milgram's test**, **Straight leg raise**, Apley scratch test and Patrick test are within normal limits.

REFLEXES

Biceps, Brachioradialis and **Patellar** are within normal limits bilaterally.

RANGE OF MOTION

Cervical ROM - estimated

Test	Measurement	Limited By	Comment
Extension	**55° (91% of normal, increased by 22%)**	Pain	

Flexion, **Left lateral flexion**, **Right lateral flexion**, **Left rotation** and **Right rotation** are within normal limits.

Lumbar ROM - estimated

Test	Measurement	Limited By	Comment
Flexion	**55° (91% of normal, increased by 37%)**	Pain	

Extension, **Left lateral flexion**, **Right ateral flexion**, **Left rotation** and Right rotation are within normal limits.

MUSCLE STRENGTH

Upper Muscle Strength

Shoulder abductors, Shoulder adductors, Shoulder flexors, Shoulder extensors, Elbow flexors and Elbow extensors are within normal limits bilaterally.

Lower Muscle Strength

Hip flexors, **Hip extensors**, **Knee extensors** and **Knee flexors** are within normal limits bilaterally.

DERMATOMES

Anterior

Left-Cervical, Right-Cervical, Left-Lumbar and Right-Lumbar are within normal limits.

ASSESSMENT

Stage of Care: Rehabilitative

DIAGNOSES

1. Radiculopathy, cervical region (M54.12)
2. Radiculopathy, lumbosacral region (M54.17)
3. Segmental and somatic dysfunction of lumbar region (M99.03)
4. Segmental and somatic dysfunction of cervical region (M99.01)
5. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
6. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
7. Disorder of ligament, vertebrae (M24.28)
8. Strain of muscle, fascia and tendon at neck level, init (S16.1XXA)
9. Sleep deprivation (Z72.820)
10. Strain of muscle, fascia and tendon of lower back, init (S39.012A)
11. Cervicalgia (M54.2)
12. Other muscle spasm (M62.838)
13. Segmental and somatic dysfunction of thoracic region (M99.02)
14. Person injured in unsp motor-vehicle accident, traffic, init (V89.2XXA)
15. Acute post-traumatic headache, intractable (G44.311)

PROGNOSIS

Good - Symptomatic and functional recovery is expected in approximately 8-12 weeks.

LONG-TERM GOALS

- Increase cervical range of motion: **Progressed from 0% to 90% of goal achieved.**

Patient is **showing improvement** since the prior visit, He is **progressing as anticipated**. Intensity is **slightly-decreased** since the prior visit.

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique was provided specifically to **C4**; **C5**; **C6**; **C7**; **T2**; **T3**; **L3**; L4; L5; Pelvis, to reduce pain reported at initial encounter by 25%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Back, to reduce pain reported at initial encounter by 25%.

- Ice/Heat (97010) consisting of ice pack was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.
- **Established Patient evaluation and management service, 99213, was provided today.**

Final exam today. The patient has reached maximum medical improvement in this office. The patient will continue with home care instructions if symptoms persist. The patient may have future office visits with other healthcare providers related to this MVC and will follow recommendations for any continued care.

Patient received care without incident.

(Electronic Signature)
Provider Name: Amanda Ruiz, DC
Date: 09/05/2024 03:40 PM

Addenda

The patient is being discharged today after a course of care with residual deficits in function from the injuries sustained from their Motor Vehicle Accident.
According to the American Medical Association (AMA) Guides to the Evaluation of Permanent Impairment 5th edition this patient may have a Permanent whole-body impairment of up to 25%.
In order to prevent further damage this patient will have to receive regular treatment at the rate of 2 times per month for life.
Approximate $66,780 in conservative lifetime treatment costs.

Lifetime Treatment Costs = [Avg. Life expectancy - Current Age] X 12 X $150= Lifetime treatment.

The average Male Life Expectancy is 75.3 years.
The average Female Life Expectancy is 81.1 years.

Electronic Signature: Ruiz, Amanda DC

Added on Sep 05, 2024